

Redacted



**Results**

**BIOPSY OR SURGICAL SPECIMEN (Order 000000)**

**Patient Information**

| Patient Name | MRN | Sex<br>Male | DOB |
|--------------|-----|-------------|-----|
|--------------|-----|-------------|-----|

**Result Information**

| Status<br>Edited | Provider Status<br>Reviewed |
|------------------|-----------------------------|
|------------------|-----------------------------|

**Entry Date**

**Component Results**

| Component<br>Surgical Pathology | Lab |
|---------------------------------|-----|
|---------------------------------|-----|

Patient Name:  
Med Rec #:  
Accession #:

Image Analysis Date Ordered:  
Date Complete:  
Date Reported:

Status: Signed Out

Interpretation  
Prognostic markers performed on block C13

**BLADDER CANCER RECURRENCE SCORE**

p21 by IHC and IMAGE QUANTITATION:

Altered  
Percent Positive Average: 0%  
Staining Intensity Grade: 0  
HistoScore: 0  
p21 altered range (<10%)

p27 by IHC and IMAGE QUANTITATION:

Unaltered  
Percent Positive Average: 78%  
Staining Intensity Grade: 2  
HistoScore: 7  
p27 altered range (<30%)

p53 by IHC and IMAGE QUANTITATION:

Altered  
Percent Positive Average: 83%  
Staining Intensity Grade: 2  
HistoScore: 7  
p53 altered range (>=10%)

ICD O-3  
Carcinoma, urothelial NOS  
8120/3  
Site: Bladder wall NOS  
C679  
J4118d14

**KI-67 by IHC and Image QUANTITATION:**

Altered  
Percent Positive Average: 35%  
Staining Intensity Grade: 2  
HistoScore: 5  
KI-67 altered range (>10%)

**Cyclin E by IHC and IMAGE QUANTITATION:**

Unaltered  
Percent Positive Average: 32%  
Staining Intensity Grade: 1  
HistoScore: 4  
Cyclin E altered range (<30%)

**INTERPRETATION:**

Prognostic Significance: Unfavorable  
3 Biomarker(s) altered  
Range: Favorable <= 2 altered biomarkers  
Unfavorable > 2 altered biomarkers

**Comment:**

In the above tumor profile the number of altered biomarkers improves the predictive accuracy for patients with PTa-T3 N0 Bladder Cancer. Greater than 2 altered biomarkers are associated with increased risk of recurrence.

**Note:**

Staining intensity based on 0 (neg) to 3+ (highest)  
Percent Positive Average: Percentage of tumor cells staining.  
HistoScore = Percentage of tumor cells staining (Scale 0-5) + Intensity (Scale 0-3).

These tests were performed IHC in conjunction with automated image analysis. The performance characteristics of the above tests have been determined in the . While some antibodies have not been approved by the FDA, clearance / approval is not mandated. These antibodies are well documented and clinically accepted prognostic indicators.

These tests should not be regarded as part of research investigations. Known positive and negative control tissue show appropriate staining.

The above tumor bio-markers were performed per        protocol.

The system used for image quantitation is the

The method used for antigen retrieval is heat induced

epitope retrieval.

College of American Pathologists (CAP) required information for predictive/prognostic markers. Type of specimen fixation and detection system: polymer detection kits are used on formalin-fixed paraffin-embedded sections. Clones used: p21 (SX118); p27(SX53G8); p53 (DO-7); Ki-67 (MIB-1); Cyclin E (HE12)

Prognostic markers performed at

Results-Comments  
{Not Entered}

#### FINAL PATHOLOGICAL DIAGNOSIS

A. Left distal ureter, biopsy:

- Negative for carcinoma or high grade dysplasia

B. Right distal ureter, biopsy:

- Negative for carcinoma or high grade dysplasia

C. Bladder and prostate, radical cystoprostatectomy:

- Invasive high grade urothelial carcinoma with micropapillary features, invasion into the prostate gland and seminal vesicles, and extensive lymphovascular invasion, pT4aN2MX
- The tumor extends to the inked and cauterized deep surgical margin multifocally
- See CAP Staging Protocol below for full report

D. Left pelvic lymph nodes, dissection:

- Four of four lymph nodes, positive for metastatic carcinoma (4/4)
- The largest nodal deposit measures 9.0 mm in greatest dimension, without extracapsular extension

#### CAP Staging Protocol for Carcinoma of the Prostate Gland

Specimen: Bladder and prostate

Procedure: Radical cystoprostatectomy

Tumor Size: Greatest dimension: 9.0 cm

Histologic Type: Urothelial (transitional cell) carcinoma, with micropapillary features

Associated Epithelial Lesions: None identified

**Histologic Grade:** Urothelial Carcinoma (WHO 2004/ISUP): High-grade

**Microscopic Tumor Extension:** Tumor involves muscularis propria  
Tumor invades perivesical tissue  
Tumor invades prostate and seminal vesicles

**Margins:** Carcinoma is present at the cauterized and inked deep margin multifocally

**Lymph-Vascular Invasion:** Present, extensive

**Pathologic Staging (pTNM):**

**Primary Tumor (pT):** pT4a: Tumor invades prostatic stroma or uterus or vagina

**Regional Lymph Nodes (pN):** pN2: Multiple regional lymph node metastasis in the true pelvis

Number of lymph nodes examined: 4, Number involved: 4

**Distant Metastasis (pM):** Not applicable

**Ancillary Studies:** If necessary, ancillary studies may be performed on block C13.

The above pathology diagnosis incorporates data elements as outlined in the CAP Cancer Protocols and Checklists which are based on the AJCC/UICC TNM, 7th edition.

\*\*\*Electronically Signed Out By [REDACTED]

#### CLINICAL HISTORY

Bladder cancer

Radical cystectomy, lymph node dissection ileal conduit, all other indicated procedures

#### SPECIMEN(S) RECEIVED

A: Left distal ureter

B: Right distal ureter

C: Bladder and prostate

D: Left pelvic lymph nodes

#### GROSS DESCRIPTION

Specimen A received fresh, labeled with the patient's name and designated "left distal ureter" is a tan, tubular segment of tissue, 0.3 cm in length x 0.5 cm in diameter. The specimen is submitted entirely for frozen section diagnosis as A1.

Specimen B received fresh, labeled with the patient's name and designated "right distal ureter" is a tan, tubular segment of tissue, dilated, 0.3 in length x 1.0 cm in diameter. The specimen is submitted entirely for frozen section diagnosis as B1.

Specimen C:

Type of specimen: Urinary bladder, ureter stumps, prostate, seminal vesicles  
The patient's name and case number on the specimen container match the

accompanying paperwork and cassettes.

Procedure: Radical cystectomy

Fixative: Fresh

Dimensions: Overall: 11.0 cm SI x 10.5 cm R-L x 7.0 cm AP

Bladder: 6.0 SI x 4.0 cm R-L x 4.0 cm AP

Right ureter: 3.0 cm length, 0.8 cm diameter

Left ureter: 2.5 cm length, 0.5 cm diameter

Prostate: 4.0 cm apex-base, 5.5 cm R-L, 3.5 cm AP

Description of findings:

Tumor: There is diffuse, ulcerated tumor covering the posterior, right lateral, and anterior wall. The ulceration extends into the left lateral mucosa. The overall tumor area is 9.0 x 4.0 x 2.0 cm. The tumor has an irregular border, extends through the muscularis propria and extends to the perivesicular inked surface. The tumor possibly extends into the seminal vesicles, with the seminal vesicles showing white, firm parenchyma. The tumor invasion through the muscularis propria into the soft tissue shows indurated, solid tumor, focally extending to the inked surface. Multiple sections are submitted for tissue procurement. The right ureteral orifice is surrounded with ulcerated, hemorrhagic mucosa. The left ureteral orifice is focally congested. The mucosa of the ureters is unremarkable.

Involvement of important adjacencies: Perivesical fat, possibly into the seminal vesicles.

Uninvolved mucosa: There is no unremarkable mucosa in the bladder.

Prostate, seminal vesicles and vasa deferentia: The prostate is serially sectioned apex to base into six slabs. The parenchyma is tan, focally cystic with pale parenchyma extending from the midline into each lobe, most prominent in the anterior aspect. In slab 6, there is focal white parenchyma along the outer edge of the prostate focally invading into the prostate parenchyma. This infiltration is most prominent in the right prostate.

Ink code: anterior bladder = black, posterior bladder = green, right prostate = orange, left prostate = blue, anterior prostate = green, posterior prostate = black

Gross photographs are taken.

Section key:

C1 - urethra margin

C2, C3 - bladder neck central to seminal vesicles, one piece

C4, C5 - right bladder neck to seminal vesicles, one piece

C6, C7 - left bladder neck to seminal vesicles, one piece

C8, C9 - right ureteral orifice, right ureter edge, one piece

C10, C11 - left ureteral orifice, left ureter edge, one piece

C12 - right bladder, tumor to inked surface

C13, C14 - right lateral wall, tumor through muscularis propria to inked surface

C15 - right lateral wall, tumor through muscularis propria to inked surface

C16, C17 - posterior wall, tumor through muscularis propria to inked surface

C18 - dome, tumor muscularis propria

C19, C20 - left bladder wall, with tumor extending from posterior wall

C21-C23 - anterior bladder, tumor through muscular propria to inked surface

C24 - slab 1 prostate right apex

C25 - slab 1 prostate left apex

C26 - slab 3 prostate right

C27 - slab 3 prostate left

C28 - slab 5 prostate right anterior

C29 - slab 5 prostate right posterior

C30 - slab 5 prostate left anterior

C31 - slab 5 prostate left posterior

C32 - slab 6 right lateral prostate anterior

C33 - slab 6 right lateral prostate posterior

C34 - slab 6 mid prostate anterior

C35 - slab 6 mid prostate posterior

C36 - slab 6 left lateral prostate anterior

C37 - slab 6 left lateral prostate posterior

Specimen D received in formalin, labeled with the patient's name and designated "left pelvic lymph node" is fragmented adipose tissue, 6.0 x 4.5 x 1.0 cm. There are four possible nodes, 0.6 cm to 4.5 x 1.2 x 0.5 cm. The cut surface of the nodes shows white-gray, slightly gritty parenchyma. The nodes are submitted entirely as follows:

D1 - one node bisected

D2 - one node serially sectioned into 5 pieces

D3 - one node quadrisected

D4, D5 - one node serially sectioned into 11 pieces

#### MICROSCOPIC DESCRIPTION

The stain quality is acceptable.

A-D. The microscopic findings are reflected in the diagnoses rendered.

#### INTRAOPERATIVE CONSULT DIAGNOSIS

Time in:            Time out:

A1-FS, Left distal ureter: "No dysplasia or carcinoma seen".

B1-FS, Right distal ureter: "No dysplasia or carcinoma seen".

Reported to            by

Interpretation performed at:

#### Results

Pathology Report - Scan [REDACTED] by [REDACTED] : Intraoperative Consultation Report-

#### Result History

BIOPSY OR SURGICAL SPECIMEN [REDACTED] - Order Result History Report.

#### Result Notes

Notes Recorded by [REDACTED]

#### Collection Information

Date and Time

Accession #

#### Lab Information

Reviewed by List

**Status:**

This order is currently not shared in

**Order**

BIOPSY OR SURGICAL SPECIMEN

(Order

**Patient Information**

Patient Name

MRN

Sex

DOB

**Visit Information**

Date

Time

Department  
Urology

Provider

**Admission Information**

Attending Provider

Admitting Provider

Admission Type

Admission Date/Time

Discharge Date

Hospital Service

Auth/Cert Status

Service Area

Unit

Room/Bed

Admission Status  
Unknown Status (No Confirmation  
Status)**Order Information**

Order Date and Time

Ordering User

Department  
Urology**Order**

Order Name

BIOPSY OR SURGICAL SPECIMEN

Code

Order Number

**Order Providers**

Authorizing Provider

Encounter Provider

**Collection Information**

Date and Time

Accession #

**Priority and Order Details**Priority  
Routine

Class

| Criteria                                             | Yes                    | No |
|------------------------------------------------------|------------------------|----|
| Diagnosis Discrepancy                                |                        | ✓  |
| Primary Tumor Site Discrepancy                       |                        | ✓  |
| HIPAA Discrepancy                                    |                        | ✓  |
| Prior Malignancy History                             |                        | ✓  |
| Dual/Synchronous Primary Noted                       |                        | ✓  |
| Case is (circle): <u>SC</u> QUALIFIED / DISQUALIFIED |                        |    |
| Reviewer Initials                                    | Date Reviewed: 1/21/14 |    |